



FAME
DENTISTRY



Supplementary Briefing Document Prepared For Remedo

MAY 2026

This brief is supplementary to your standard onboarding questionnaire which we have completed separately. It covers the strategic decisions flagged in our 28 April call as the ones that make this project different from a typical dental website. Specifically: the different audiences we have to serve, the referring-dentist positioning angle, the role of NPS as a live brand element, the lead-magnet integration logic.

TL;DR – what this brief argues

1. This is a three-audience site, not a one-audience site. Business leaders, general consumers and referring dentists. Each needs to find what they came for without alienating the others.
2. The single biggest positioning lever we have is peer credibility. “The dentists that other dentists refer their patients to” ... “the dentists other dentists choose for themselves and their families” ... “the teachers/mentors that other dentists go to when they want to improve their skills/capabilities.
3. Ferhan’s dual qualification (medicine + dentistry) opens an editorial territory no competitor can occupy. It feeds Insights, the podcast, and AEO presence on queries no other dentist is competing for.
4. NPS is not a static claim. It’s a live, displayed, dated, methodologically transparent feature of the site. That’s the difference between a USP and a marketing line.
5. Mimetic marketing logic translates into specific design choices. Named credentialled testimonials, visible alliance partners, an editorial register a CEO would forward to a peer.
6. Two lead magnets, two distinct flows. First Impressions, Final Decisions (paperback, physical address capture, business-leader audience). White Knight Report/7 Costly Mistakes (PDF, email capture, consumer audience).

1. Why this project differs from a typical dental website

Three structural differences from a standard private-practice site:

1. Multi-audience architecture. We have different audiences (not one audience with sub-segments) and that means different Jobs To Be Done.
2. Existing strategic asset base. Book, FAME recruitment manifesto, lead magnets, NPS positioning, branded collateral.

3. NPS-as-USP positioning. The site has to function as ongoing live demonstration of the claim, not a static restatement.

The implication for design: frameworks normally applied to typical brochure-led dental sites are largely inappropriate to our requirements. We are asking you to do somewhat unfamiliar work. We know that, and we will endeavour to support your requests for clarity and decision in a timely manner, likewise providing copy from your direction, that doesn't hold you up.

2. The three audiences

Each audience has a job-to-be-done, a tone, and an entry point on the site. The standard dental website assumes one audience and trades cleanly between "trust me with your family" and "look at our nice waiting room." We have a substantially harder job, and a substantially bigger payoff to be had by figuring out how to get it right.

Audience	What they need from the site	Tone / register	Primary entry
Business leaders & entrepreneurs	Confidence in privacy, time, and standards. Concierge flow. A feeling of membership/exclusivity	Editorial, restrained, peer-to-peer	Concierge/Member exclusivity
General consumers (implants & cosmetic) Relational NOT Transactional patients	Trust in the work itself. Reassurance against bad alternatives (Turkey, same-day teeth). Peer-referral signal.	Warm but authoritative; explanatory	Implants = 7 Costly Mistakes Cosmetic = to be decided in consultation with Tariq
Referring dentists	Confidence that the patient comes back to them. Confidence that referring FAME will make them look good.	Peer-professional; technical without being cold	For Referring Dentists/case studies

The hardest audience to design for is the referring dentist. They're reading the same homepage as a Morgan Stanley MD. The site needs to communicate "we are operating at the top of this market" (which the MD needs to see) AND "we will return your patient to

you" (which the referring dentist needs to see). Through use of the right language there should be no tension between these two messages.

3. Site architecture

Our original branding pack identified a four-domain ecosystem: famedentistry.co.uk (consumer), referrals.famedentistry.co.uk (referring dentists), training.famedentistry.co.uk (dentist training), and lab.famedentistry.co.uk (lab work). Building all four at quality in eight weeks is unrealistic.

For the initial development (stage 1) we will focus entirely on famedentistry.co.uk

4. The referring-dentist positioning angle

A large proportion of Ferhan's and Tariq's current work – and a significant share of expected FAME revenue – comes from referrals from other dentists. This is also one of the most powerful trust signals available to us. Three reasons it matters for the homepage, not just the peer-facing page:

1. **The White Knight Positioning Report opens with 'Mistake #1:** Judging a practice by its own marketing.' It tells consumer readers to ask: 'What percentage of your implant patients are referred by other dentists?' The website should volunteer the answer before the question is asked.
2. **Peer endorsement is the most credible third-party signal.** Other dentists put their own reputations on the line every time they refer. That's a far higher signal-to-noise ratio than any consumer review.
3. **It is uniquely defensible.** Almost no practice can claim peer-referral volume at this scale. It is a moat.

This positioning should be presented confidently on the home page and linked through to:

- **Implants page** – expanded language: 'Most of our complex implant patients are referred to us by other dentists. We think that's the only credential that matters.'
- **Referrals page** – dedicated peer-facing register: how cases are handled, how we communicate during treatment, the patient-return protocol, the lab options and reassurance that the patient remains their patient.

- **Founders page** – named referring dentists' testimonials (with permission).

5. The dentists-other-dentists-turn-to angle

Both Ferhan and Tariq have trained or mentored hundreds of UK dentists. This is currently invisible online. It's a credentialling argument that no other dental practice in Scotland can make.

The credentialling logic is simple: If the people who train other dentists chose this practice, why would you go anywhere else?

Where it appears:

- Founders page – with specific evidence: programme names, numbers, course alumni count.
- A 'Training' entry in the navigation – in phase 1 this links to a single training-overview page; in a later phase of development it links to training.famedentistry.co.uk.
- Insights/Journal – articles aimed at clinicians appearing alongside articles aimed at consumers, signalling the dual audience.

6. Ferhan's dual qualification – The Pre-Surgical Optimisation Angle

Ferhan's dual medical and dental qualification creates an opportunity to claim a distinctive, defensible position that very few practitioners can credibly occupy: the medical optimisation of patients ahead of dental surgery.

This is specific pre-surgical service, involving tests not commonly carried out in a dental practice to prepare the patient medically before any procedure. Ferhan is uniquely placed to deliver because of his clinical training.

This is a valuable positioning angle:

Pre-surgical optimisation is a largely unoccupied territory.

The differentiation shows up directly in the treatment journey: tests, considerations and conversations that other practices simply do not provide. The patient conclusion "this practice does things others don't" and legitimises the tagline: 'A Decidedly Different Dental Experience'

This innovative approach should resonate well with intelligent, performance-minded professionals

A defined clinical claim of this kind opens up collaborative opportunities with medical-device firms, diagnostic companies, surgical training providers etc

The practical application:

A unique named service line on the website – Pre-Surgical Patient Optimisation

Content authority. The podcast with Tariq would be the perfect educational format to introduce this offering.

Excellent PR and professional positioning.

The unique claim here is that Ferhan prepares and optimises patients medically before he treats them surgically.

7. NPS as a live brand element, not a static claim

FAME's vision is to claim the world's highest NPS in dentistry within three years of opening. That cannot live as copy on the homepage and nowhere else. The site should treat NPS as an ongoing displayed brand artefact:

- Display the current NPS score prominently – updated quarterly at minimum, monthly if the workflow is clean.
- Show the trend over time. A simple line chart of quarterly scores from opening day onward.
- Link to a methodology page – how we measure, how often, who collects, response rate, full transparency. This is the credibility ballast that turns a marketing claim into a scientific one.
- Include patient verbatims as quoted 'why I gave a 10' responses, with date and (where consented) name and role.
- Treat each milestone as a press hook – highest in Scotland, highest in the UK, highest in Europe, highest in the world. The site's NPS section is the source of truth that journalists can cite.

Question for your team: how cleanly can you wire a live or near-live NPS feed into Webflow? There are various NPS software options. From a usability point of view our current preference would be customerthermometer.com

8. The first two lead magnets

We have two lead magnets in production, aimed at different audiences and different points in the buyer journey. They will need different treatments on the site.

Asset	Audience	Format	Capture	Buyer stage
First Impressions, Final Decisions	Business leaders	Paperback (mailed)	Postal address	Awareness – they don't yet know they need a dentist
7 Costly Mistakes/White Knight Positioning Report	General consumers researching implants	PDF	Email	Stages 2–3 – they know they need implants, choosing a provider

We envisage both lead magnets will have dedicated landing pages with referenceable from the homepage. The paperback request flow needs to feel materially different from the PDF – more considered, more personal. We are putting Ferhan's signed letter inside the paperback. The web copy should reflect the offline experience.

AEO consideration: each 'mistake' in the White Knight report could become its own AEO-friendly article. The report could be deconstruct into 7 long-form pages plus an overview, all schema-marked, all targeting Stage 2-3 implant buyer queries.

9. Mimetic marketing – implications for design

The strategic logic underneath the FAME positioning is mimetic: business leaders signal status by being in places, holding things, and using services that other business leaders endorse. That logic translates into specific design and content choices that go beyond 'premium feel.'

- Testimonials are from named, credentialled business figures – not anonymous 'Sarah, 52.' With permission, role and company appear. 'James Macleod, CFO, [Company]' substantially outperforms 'Sarah, business owner.'
- Featured patients are people the audience would recognise or find relatable as peers. We will not be using model photography.

- Editorial coverage and press mentions are visible – Scottish Dental Magazine, business press – once secured. A dedicated ‘As featured in’ section, treated with restraint.
- Strategic alliance partners (HSBC if/when confirmed) appear cleanly co-branded on the relevant pages. The presence of a recognised partner does mimetic work the practice cannot do alone.
- The site itself should feel like something a CEO would forward to a peer. If a Morgan Stanley MD wouldn’t share this URL with another MD, we’ve missed the brief.

The mimetic test: if you remove the FAME branding from the page and replace it with a fictional private bank, would the page still feel right? If it suddenly looks dental generic, we need to bring back the editorial substance, not the branding.

10. Strategic alliance with HSBC (or similar)

Pre-decisional, but worth designing for. If confirmed, an alliance affects:

- The book sponsorship and distribution mechanic – specifically, who the book reads as being ‘from’ (Ferhan only, vs Ferhan + sponsor partner).
- A potential co-branded presence on the First Impressions, Final Decisions landing page.
- Possibly a co-hosted launch event that the site needs to accommodate – either an events page in the navigation, or a specific event landing.

Recommendation: design with the assumption that a partner co-brand may need to slot in cleanly without breaking the visual hierarchy. A reserved area on the book landing page and a flexible ‘As featured in/In partnership with’ strip pattern that can carry a logo without screaming about it.

11. AI/AEO content strategy

Your recommendations and our thinking are in alignment: AEO is won by answering the questions our audience is asking the AI engines. We would like to gain ai reference/recommendation for:

- Business leaders
- Consumer implant patients
- Cosmetic patients

- Referring dentists
- (A later phase) Dentists seeking training/mentorship

We're thinking that The White Knight Positioning Report represents a perfect piece of AEO content. Each 'mistake' could exist as its own article, FAQ-marked, internally linked. The Implant Buyer Stages document defines the awareness levels the content should write to.

12. Photography and video – a brief request

We will commission the photography and video. We need from you:

1. A shot list aligned with the design direction, so we don't shoot the wrong things.
2. Aspect ratios and resolutions for each placement on the site.
3. Recommendations on photographer style references at mood-board level.
4. A view on whether stills are sufficient for v1, or whether short loop video is needed for the hero.

Likely subjects:

- Building exterior on Scotland Street – architectural, multiple times of day, multiple weather conditions.
- Reception, consultation rooms, treatment rooms, training suite – interior, editorial register.
- Founder portraits – Ferhan and Tariq, individually and paired, in the practice and in environments that read as 'business leader who happens to be a dentist.'
- A documentary sequence – a treatment plan being built, a CBCT scan being read, a referring-dentist communication being sent.
- Concierge moments – arrival, the consultation suite at desk-height, a private vehicle (we may need to stage this for v1).

We will not be shooting patients in chairs, or 'smile reveal' sequences, for v1.

13. What we're asking from Remedo next

Before we move to design directions:

1. A proposed sitemap and information architecture document for Phase 1, given the audiences and page list above.

2. Three design direction concepts, NOT five colour variations on the same template (per Harsh's own observation that the iteration loop is unproductive after the first round).
3. A page-by-page brief template that we'll fill in with copy.
4. A photo and video shot list with aspect-ratio specifications for each placement.
5. A confirmed AEO content plan: which pages need FAQ schema, what queries each page targets, recommended article topics for launch content.
6. Clear identification of the dependencies from our side.

We are responsive. We will not be a slow client. We do have a strong opinion on what we're trying to build, and we'd rather make decisions with you than be presented with options.

This document references:

- The Implant Buyer Stages document (FAME_Implant_Buyer_Stages)
- Book: First Impressions, Final Decisions (First_Impressions_Final_Decisions_2026_02_04)
- White Knight Positioning Report/7 Costly Mistakes (FAME_White_Knight_Positioning_Report_2026_04_14)
- FAME Recruitment Manifesto (FAME_Recruitment_Manifesto_May_2026)